Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0007	**Age / Sex**	42 / F
Setting	Emergency Dept	**Primary dx**	Generalized tonic-clonic seizure, post-ictal state

Chief complaint

Generalized seizure at home, not fully awake since.

History of present illness

Witnessed by husband: sudden stiffening and rhythmic jerking of all limbs for approx 3 min, tongue biting, urinary incontinence. Second seizure en route lasting 2 min. Missed levetiracetam doses over 3 days after running out.

Past history / medications

Epilepsy since age 20 (focal to bilateral tonic-clonic), levetiracetam 1000 mg BD. No head injury recently.

Examination

Temp 37.1 C, HR 112, BP 128/76. Lateral tongue laceration. Pupils equal, reactive. No meningism. Plantar responses extensor bilaterally in the post-ictal period.

Glasgow Coma Scale

Eyes open to pain. Mumbles incomprehensibly. Motor: withdraws from pain.

GCS on arrival (post-ictal)	Score	Response
Eye opening (E)	2	To pain
Verbal (V)	2	2 - Incomprehensible sounds
Motor (M)	4	Withdraws from pain
TOTAL GCS		**8**

GCS at 90 min	Score	Response
Eye opening (E)	4	Spontaneous
Verbal (V)	4	4 - Confused
Motor (M)	6	Obeys commands
TOTAL GCS		**14**

Investigations

Glucose 6.0 mmol/L, sodium 138, calcium normal, lactate 5.2 (falling). CT head normal. Levetiracetam level undetectable.

Assessment

Breakthrough tonic-clonic seizures from antiepileptic non-adherence with prolonged post-ictal state (GCS 8), no evidence of ongoing status epilepticus.

Plan / disposition

Levetiracetam IV load then oral restart. No benzodiazepine needed after cessation. GCS 14 at 90 min, 15 at 4 h. Neurology follow-up, driving advice, prescription reconciliation.